Country Profile: Australia



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Table of Contents

Country Overview 3

Population and Demographics 3

Government 4

Economy 4

Disease Prevalence 5

Australia Health System 6

Public Healthcare System 6

Private Healthcare System 6

Healthcare Financing 7

Workforce 8

Education 9

Healthcare Outlook 9

Competitive Landscape 9

Recent Developments 11

CSI Assessment 12

Country Overview

Population and Demographics

Capital	Canberra[1]	**National language** Legal status, not usage; 72.0% of people spoke only English at home at the 2021 Census	English, the de facto national language; no official language is set in law[2]
Population Estimated resident population, all residents	27,921,150 (31 March 2026)[3]	**Annual population growth** Natural increase 100,600 plus net overseas migration 292,100	1.4% (year to 31 March 2026)[3]
Area Total area, mainland and islands	7,688,287 sq km[4]	**GDP (PPP), total** PPP, current international dollars	About 1.99 trillion international dollars (2025)[5]
GDP (PPP), per capita PPP, current international dollars	About 71,930 international dollars (2025)[6]	**GDP (nominal), total** Current US dollars at market exchange rates	About US$1.80 trillion (2025)[7]
Government type	Federal parliamentary constitutional monarchy[1]	**Head of state**	King Charles III, represented by Governor-General Sam Mostyn[1]
Head of government	Prime Minister Anthony Albanese[8]	**Currency**	Australian dollar[9]
Urban population Share of total population, World Bank (UN-based) definition; ABS counts 18,750,888 people in capital cities at 30 June 2025	87.7% (2025)[10]	**Population aged 65 and over** Derived from ABS estimated resident population by age: 15.5% in the capital cities and 21.9% in the rest of Australia, weighted by population	About 17.5% (30 June 2025)[11]
Population aged 65 and over (World Bank) Share of total population, UN-based estimate; ABS projection base year 2022 was 17%	18.1% (2025)[12]	**Total fertility rate** Births per woman	1.48 births per woman (2024)[13]
Life expectancy at birth ABS life tables, three-year period	81.1 years for males and 85.1 years for females (2022-2024)[14]	**Life expectancy at birth (World Bank)** Both sexes; equals the 2023 value, so the 2024 figure may be carried forward	83.1 years (2024)[15]
Born overseas Share of the Census population	27.6% (2021 Census)[16]	**Median age** Years, ABS Greater Capital City areas against the rest of Australia	37.0 in capital cities, 42.1 elsewhere (30 June 2025)[11]

Australia's estimated resident population was 27,921,150 at 31 March 2026, after annual growth of 392,700 people (1.4 percent) made up of natural increase of 100,600 and net overseas migration of 292,100.[3] Migration accounts for most of this growth. At the 2021 Census, 27.6 percent of residents were born overseas, and the leading languages used at home besides English were Mandarin, Arabic, Vietnamese, Cantonese and Punjabi.[16] The country is highly urban: the capital cities held 18,750,888 people at 30 June 2025, and the World Bank puts the urban share at 87.7 percent in 2025.[10,17] The median age was 37.0 years in the capital cities and 42.1 years in the rest of Australia.[11] People aged 65 and over were about 17.5 percent of the population at 30 June 2025 on ABS age data and 18.1 percent in 2025 on World Bank data, which rests on a UN estimate; the ABS projects 25 to 27 percent by 2071.[11,12,18] Fertility was 1.48 births per woman in 2024.[13] For health services, an older population and a wide gap between city and regional age profiles raise demand for chronic care and make access uneven.

Government

Australia is a federal parliamentary democracy and a constitutional monarchy.[1] Governor-General Sam Mostyn, the 28th holder of the office, represents the monarch, King Charles, and gives Royal Assent to bills on his behalf; the Australian Prime Minister recommends the Governor-General to the King for appointment.[1] At the 2025 federal election the Australian Labor Party won 94 seats in the House of Representatives, the Liberal/National Coalition 43 and independents 10, with the Greens, Katter's Australian Party and Centre Alliance holding one each.[19] The House stays at 150 seats, and the next federal election is due by 2028.[20] Anthony Albanese is Prime Minister.[8] Responsibility for health is shared. The Commonwealth funds Medicare, the national scheme that pays for private medical services, while the states and territories run public hospitals, which the Commonwealth supports through the National Health Reform Agreement.[8,21] Commonwealth hospital funding was agreed in January 2026 for 2026-27 to 2030-31.[8]

Economy

Australia is a high-income economy, with GDP of about US$1.80 trillion at market exchange rates in 2025, or about 1.99 trillion international dollars at purchasing power parity.[5,7] The two totals measure the same economy on different bases and should not be compared with each other. GDP rose 0.4 percent in the June quarter 2026 and 2.1 percent over the year, in chain volume terms.[22] The unemployment rate was 4.7 percent in September 2026 (seasonally adjusted).[23] Inflation has turned up: the consumer price index rose 4.3 percent in the 12 months to September 2026, and trimmed mean inflation was 3.6 percent.[24] The Reserve Bank raised its cash rate target by 0.25 percentage points to 4.85 percent effective 4 November 2026, its fifth increase of the year, from 3.60 percent at the end of 2025.[25] Higher borrowing costs and prices put pressure on hospital operators and household budgets, which affects private insurance uptake and elective demand.

Disease Prevalence

Deaths registered All registered deaths, by date of registration	188,421 (2025, preliminary)[26]	**Age-standardized death rate** Adjusted for age structure and population size	495.5 per 100,000 (2025)[26]
Leading cause of death Share of all registered deaths	Dementia, including Alzheimer's disease, 10.1% of deaths (2025)[26]	**Second leading cause of death** Share of all registered deaths	Ischemic heart diseases, 8.6% of deaths (2025)[26]
Other leading causes (2025)	Cerebrovascular diseases, lung cancer, chronic lower respiratory diseases[26]	**Top five causes combined** Share of all registered deaths	33.3% of deaths (2025)[26]
Largest disease burden Share of total burden across 17 disease and injury groups, AIHW	Cancer, 16.4% of disability-adjusted life years (2024)[27]	**People with a chronic condition** Share of people living with at least one chronic condition, AIHW	61% (Australia's health 2026)[28]
Adults overweight or obese AIHW 2022-24 estimate, 13.2 million adults; ABS National Health Survey 2022 measured adults	67% (2022-24)[28]; 65.8% (2022)[29]	**Adults who smoke daily** Share of adults aged 18 and over, ABS National Health Survey	10.6% (2022)[30]; 22.4% in 2001[30]
People aged 14 and over who smoke daily AIHW National Drug Strategy Household Survey 2025; 5.8% among adults aged 18 and over	5.6% (2025)[31]	**Daily smokers, OECD comparison** People aged 15 and over, OECD Health at a Glance 2025, based on 2022-23 survey data	8.5% (OECD average 14.8%)[32]
Adults who have used e-cigarettes Share of adults who have ever used e-cigarette or vaping devices	14.4% (2022)[30]	**People with diabetes** Share of people with diabetes, ABS National Health Survey	5.3%, about 1.3 million (2022)[33]
Deaths from noncommunicable diseases Share of total deaths, World Bank	89.6% (2021)[34]	**Alcohol consumption** Liters of pure alcohol per person, OECD Health Statistics 2025	10.5 liters per person aged 15 and over (OECD average 8.5)[32]

The Australian Bureau of Statistics registered 188,421 deaths in 2025, a preliminary count, and the age-standardized death rate fell to 495.5 per 100,000.[26] Dementia was the leading cause at 10.1 percent of deaths, the first year it passed 10 percent, ahead of ischemic heart diseases at 8.6 percent, with cerebrovascular diseases, lung cancer and chronic lower respiratory diseases completing the top five.[26] Cancer causes the largest share of disease burden at 16.4 percent of disability-adjusted life years in 2024.[27] Overweight and obesity have overtaken tobacco as the leading risk factor, affecting 67 percent of adults in 2022-24, and 61 percent of people live with a chronic condition.[28] The ABS measured 65.8 percent of adults as overweight or obese in 2022, a different year and method from the AIHW estimate.[29] Daily smoking fell to 10.6 percent of adults in 2022, from 22.4 percent in 2001, and the AIHW's 2025 household survey found 5.6

percent of people aged 14 and over smoking daily; the surveys differ in age range and method.[30,31] The World Bank attributes 89.6 percent of deaths to noncommunicable diseases in 2021.[34]

Australia Health System

Public Healthcare System

Australia covers its whole population for a core set of services through Medicare, and mandatory prepayment financed 73 percent of health spending in the OECD's latest comparison.[32] Public hospitals are run by the states and territories with Commonwealth funding. In 2023-24 there were 704 public hospitals with 67,280 available beds, about 2.5 per 1,000 people, and recurrent spending of A$103.1 billion (US$72.3 billion), 94 percent of it from governments.[21] Public emergency departments recorded 9,094,312 presentations in 2024-25.[21] The Pharmaceutical Benefits Scheme subsidizes the cost of medicines for most medical conditions.[35] In January 2026 National Cabinet agreed a five-year funding deal under which the Commonwealth will provide a record A$219.6 billion (US$154 billion) to state-run public hospitals from 2026-27 to 2030-31, and the states agreed to deliver some disability services outside the National Disability Insurance Scheme.[8,36] In primary care, the bulk billing incentive was extended to all patients on 1 November 2025.[37] GP bulk billing reached 84.1 percent in April to June 2026, up 4.9 points on a year earlier, after 81.4 percent in November 2025 to January 2026.[38,39] The government's target is 90 percent by 2030.[39]

Private Healthcare System

Private insurance is widely held and subsidized. At 30 June 2026, 12,823,752 people, or 45.8 percent of the population, had hospital treatment cover, and 55.5 percent had general treatment cover.[40] Insurers paid A$5,330.86 million (US$3.74 billion) in hospital benefits in the June 2026 quarter, and 1,209,359 people were subject to a Lifetime Health Cover loading.[40] Premiums are community rated, so all policy holders pay the same for the same policy regardless of age or health, and the government encourages uptake through a premium rebate and the Medicare Levy Surcharge on higher earners without hospital cover.[41] The declared-hospital list held 641 private hospitals, including day surgeries, at 17 June 2026, and private hospitals admitted 5,125,152 patients in 2024-25, about 40 percent of all admissions, according to the industry body citing AIHW data.[42,43] No current count of private beds is published. Ramsay Health Care is the largest operator, with 70 hospitals, clinics and day surgery units in Australia.[44] Healthscope, the second largest, entered receivership in May 2025.[45] A department review in late 2024 found that average profitability in the sector had declined and that several hospitals were at high risk of near-term exit.[41]

Healthcare Financing

Total health spending, 2024-25 AIHW national health accounts, financial year 2024-25, health goods and services; 2023-24 was A$270.5 billion (10.1%)	A$290.7 billion (US$204 billion), 10.5% of GDP[46]	**Health spending per person, 2024-25** AIHW, all sources of funding	A$10,614 (US$7,440)[46]
Current health expenditure, share of GDP World Bank, calendar year, current health expenditure	10.40% (2023); 8.67% in 2012[47]	**Health spending, share of GDP (OECD)** OECD Health at a Glance 2025, simple OECD average; the AIHW ratio applies OECD definitions and is not comparable with its national figure	10.3% (2024); OECD average 9.3%[48]; AIHW OECD-basis ratio 10.7% (2024)[46]
Current health expenditure per capita World Bank, current US dollars	US$6,980 (2023, current US$)[49]	**Health spending per person (OECD)** OECD, US dollars adjusted for purchasing power parity	US$7,469 (2024, PPP); OECD average US$5,967[48]
Government share of spending Commonwealth A$113.9 billion (US$79.9 billion) plus states and territories A$89.6 billion (US$62.8 billion), share of A$290.7 billion	About 70% (2024-25)[46]	**Government share of current health expenditure** World Bank, domestic general government health expenditure	73.7% (2023)[50]
Out-of-pocket spending, 2024-25 AIHW, spending by individuals	About A$46 billion (US$32.3 billion), A$1,693 (US$1,190) per person[46]	**Out-of-pocket share of current health expenditure** World Bank, share of current health expenditure	15.8% (2023); 19.9% in 2012[51]
Comparison, current health expenditure share of GDP World Bank, latest year available for each; OECD members is the World Bank aggregate	Australia 10.40% (2023), New Zealand 10.06% (2024), United Kingdom 11.13% (2024), Canada 11.31% (2024), OECD members 12.75% (2023), United States 16.69% (2023)[52]		

Health spending was A$290.7 billion (US$204 billion) in 2024-25, or 10.5 percent of GDP and A$10,614 (US$7,440) per person, up from A$270.5 billion and 10.1 percent in 2023-24, according to the Australian Institute of Health and Welfare.[46] Governments paid 70 percent, with the Commonwealth contributing A$113.9 billion (US$79.9 billion) and the states and territories A$89.6 billion (US$62.8 billion).[46] Individuals paid about A$46 billion (US$32.3 billion) out of pocket, or A$1,693 (US$1,190) per person.[46] Other measures use different definitions. The World Bank reports current health expenditure of 10.4 percent of GDP for 2023, with out-of-pocket spending at 15.8 percent of it, down from 19.9 percent in 2012.[47,51] The OECD's Health at a Glance 2025 shows 10.3 percent for 2024, while the AIHW's own OECD-basis ratio is 10.7 percent for 2024.[46,48] Australia spends more per person than the OECD average.[48] The World Bank's OECD members

aggregate for 2023 (12.75 percent of GDP) differs from the OECD's own average of 9.3 percent, so the two should not be mixed.[48,52]

Workforce

Physicians per 1,000 people Practising physicians, OECD Health Statistics 2025	4.2 (2023); OECD average 3.9[48]	**Physicians per 1,000 people (World Bank)** World Bank series, an older year than the OECD value	4.09 (2022); 3.29 in 2012[53]
Nurses per 1,000 people Practising nurses, OECD Health Statistics 2025	13.0 (2023); OECD average 9.2[48]	**Nurses and midwives per 1,000 people (World Bank)** World Bank series, an older year than the OECD value	13.45 (2022)[54]
Registered and employed medical practitioners Headcount registered and employed in medicine, Health Workforce Data; not comparable with OECD practising physicians per 1,000	126,206 (2025); 128,023 full-time equivalent[55]	**Employed nurses and midwives** Headcount employed in nursing and midwifery, Health Workforce Data; not comparable with OECD practising nurses per 1,000	421,796 (2025); 384,252 full-time equivalent[56]
Hospital beds per 1,000 people, all hospitals All hospital beds, public and private, OECD 2025 edition; OECD notes Australian data cover 2016 to 2022 and the World Bank shows 3.82 for 2016	3.8; OECD average 4.2[48]	**Public hospital beds** Average available beds for admitted patients in public hospitals only; excludes private hospitals	About 68,200, 2.5 per 1,000 people (2024-25)[57]; 67,280 in 2023-24[21]
Registered health practitioners All professions registered with Ahpra	999,105 (2025/26), up 4.1%[58]	**First-time registrants** All professions, Ahpra annual report 2025/26	70,832 (2025/26), over 23,000 internationally qualified[58]

Australia has more doctors and nurses per head than the OECD average. The OECD counts 4.2 practising physicians and 13.0 practising nurses per 1,000 people for 2023, against OECD averages of 3.9 and 9.2.[48] National registers are newer: 126,206 registered and employed medical practitioners (128,023 full-time equivalent) and 421,796 employed nurses and midwives (384,252 full-time equivalent) in 2025.[55,56] The World Bank series shows 4.09 physicians for 2022.[53] On the OECD's measure hospital capacity is similar to the OECD average, 3.8 beds per 1,000 against 4.2, though the OECD says Australian data cover 2016 to 2022.[48] Public hospitals alone had about 68,200 available beds in 2024-25, or 2.5 per 1,000 people.[57] The two bed measures count different things and should not be compared directly. Ahpra registered 999,105 health practitioners in 2025/26, up 4.1 percent, and 70,832 people registered for the first time, including more than 23,000 who trained overseas.[58]

Education

The Australian Medical Council accredits the primary medical programs of 24 education providers, 22 in Australia and two in New Zealand, which award Doctor of Medicine and bachelor of medicine and surgery degrees, including combined bachelor and doctoral pathways.[59] All medical graduates must complete an internship before general registration with the Medical Board of Australia, and interns hold provisional registration during that first postgraduate year.[60] The Council also accredits the specialist medical colleges that run postgraduate training.[61] Overseas-trained doctors are a large part of the intake. Ahpra reported more than 26,700 internationally qualified practitioners among 69,880 first-time registrants in 2024/25, and more than 23,000 among 70,832 in 2025/26, as faster pathways for internationally qualified doctors and nurses were introduced while the Kruk review was implemented.[58,62] Retention is a concern: an Ahpra study found that 79 percent of surveyed practitioners intended to keep their registration, while more than 12 percent were unsure or intended to leave, citing burnout, retirement and feeling undervalued.[62]

Healthcare Outlook

Demand will keep rising as the population ages: people aged 65 and over were about 17.5 percent of residents at 30 June 2025 on ABS data (18.1 percent on World Bank figures), and the ABS projects 25 to 27 percent by 2071.[11,12,18] Public funding is set for the medium term. The Commonwealth will provide a record A$219.6 billion (US$154 billion) to state-run hospitals from 2026-27 to 2030-31, and the 2026-27 Budget restated the A$25 billion (US$17.5 billion) of additional hospital funding within that agreement and provided A$1.8 billion (US$1.26 billion) to make the 137 Medicare Urgent Care Clinics permanent.[8,63] Primary care is the other focus, with a 90 percent bulk billing target for 2030.[39] Private providers are under strain. A department review found falling profitability and several hospitals at high risk of near-term exit, Healthscope's 25 hospitals are being split among four operators, and the cash rate reached 4.85 percent on 4 November 2026.[25,41,45] The department has also analyzed changes to the private health insurance rebate, which it calls poorly targeted toward older Australians.[41] Workforce retention is a further risk, with more than 12 percent of surveyed practitioners unsure about staying or planning to leave.[62]

Competitive Landscape

Foreign health system ties in Australia are mainly research and training collaborations. Cleveland Clinic runs a two-year colorectal fellowship with Epworth HealthCare, and Emory University and the University of Queensland established the Queensland Emory Vaccine Centre.[64,65] Boston Children's Hospital, SickKids and Great Ormond Street work with Murdoch Children's Research Institute on precision child health.[66,67] Healthscope entered receivership in May 2025 after its North American private equity owner, Brookfield, withdrew support.[45]



Active foreign health-system partnerships by city of the partner in Australia. Source: CSI partnerships tracker, verified 2026-10-20. Boundaries: Natural Earth; not an official depiction.

Confirmed active partnerships by institution

Institution	Based in	Nature of collaboration	Latest evidence
US academic medical centers			
Boston Children's Hospital	United States	Research collaboration	2025-02[68]
Cleveland Clinic	United States	Training and education	2025-11[64]
Emory University	United States	Research collaboration	2025-01[65]
Memorial Sloan Kettering Cancer Center	United States	Research collaboration	2026-07[69]
Ochsner Health	United States	Training and education	2025-12[70]
Weill Cornell Medicine	United States	Research collaboration	2026-09[71]
Other foreign health systems			
Apollo Hospitals	India	Research collaboration	2024-11[72]

Institution	Based in	Nature of collaboration	Latest evidence
Great Ormond Street Hospital and UCL Great Ormond Street Institute of Child Health	United Kingdom	Research collaboration	2025-02[66]
Ho Chi Minh City Eye Hospital	Vietnam	Training and education	2026-04[73]
Hokkaido University (Faculty of Health Sciences)	Japan	Training and education	2025-07[74]
Manipal Academy of Higher Education (MAHE)	India	Research collaboration	2026-09[75]
Max Healthcare	India	Research collaboration	2025-12[76]
Sichuan Provincial People's Hospital	China	Research collaboration	2025-04[77]
The Chinese Medicine Hospital of Hong Kong	China (Hong Kong)	Research collaboration	2025-06[78]
The Christie NHS Foundation Trust	United Kingdom	Training and education	2025-11[79]
The Hospital for Sick Children (SickKids)	Canada	Research collaboration	2025-02[66]
University of Galway (BioInnovate Ireland)	Ireland	Research collaboration	2026-07[80]
West China Hospital, Sichuan University	China	Research collaboration	2025-04[81]

Only partnerships with dated evidence of activity in the last 24 months are listed. Unclear and recently ended partnerships are in the CSI partnerships tracker.

Australia is a small destination for international patients and a larger source of them. A 2019 trade review described inbound medical travel as a few wealthy Asia-Pacific patients; no official count exists, and Epworth's figure of about 600 international patients a year includes visitors who fell ill.[82] About 15,000 Australians travel overseas for treatment each year, according to the Victorian government, and some diaspora patients choose surgery in China or Taiwan.[83,84] For a US academic center, Australia is mainly a market for partnerships, training and research, not referrals.

Recent Developments

2026-10-30 Ahpra reports a record workforce and record notifications

Ahpra's 2025/26 annual report shows registered health practitioners rose 4.1 percent to 999,105.[58] There were 70,832 first-time registrants, including more than 23,000 internationally qualified practitioners, after new expedited pathways for international practitioners.[58] Ahpra also completed a record 13,171 notifications.[58]

2026-09-14 Healthscope's hospitals to be split among four operators

Receivers announced that Healthscope's 25 hospitals and central services platform will be divided among four operators, with Calvary Health Care taking 14 and Healthe Care, ACURIO Health and KnG Group sharing the rest.[45] Healthscope, the second-largest private hospital operator, entered receivership in May 2025.[45]

2026-05-12 Budget 2026-27 confirms hospital funding and secures urgent care clinics

The Budget restates the A$25 billion (US$17.5 billion) of extra public hospital funding agreed in January, for a record A$220.3 billion (US$154 billion) over five years.[63] It gives A$1.8 billion (US$1.26 billion) to make 137 Medicare Urgent Care Clinics permanent and A$5.9 billion (US$4.14 billion) for new PBS listings.[63,85]

2026-01-30 National Cabinet agrees a five-year public hospital funding deal

The Commonwealth will provide a record A$219.6 billion (US$154 billion) to state-run public hospitals from 2026-27 to 2030-31, including A$25 billion (US$17.5 billion) in additional funding.[8] In return the states and territories will play a greater role in providing disability support outside the NDIS.[36]

2025-11-01 Bulk billing incentive extended to all patients

From 1 November 2025 the Medicare bulk billing incentive applies to any patient, not only children and concession card holders, as part of a A$7.9 billion (US$5.54 billion) investment.[37] GP bulk billing reached 84.1 percent in April to June 2026, up 4.9 points on a year earlier.[38]

CSI Assessment

Everything below comes from Cedars-Sinai's internal data (patient referral records, finance, relationship history) and human judgment, not from the research agents. Fields left blank are marked and must be completed by the analyst before this profile is used.

Patient Volume & Revenue (most recent FY)
[MANUAL INPUT REQUIRED]

Referral Sources
[MANUAL INPUT REQUIRED]

Relationship History
[MANUAL INPUT REQUIRED]

Complications
[MANUAL INPUT REQUIRED]

Key Contacts
[MANUAL INPUT REQUIRED]

Opportunities
[MANUAL INPUT REQUIRED]

CSI Recommendation
[MANUAL INPUT REQUIRED]